

CITATION: Rajic v. Atkins, 2011 ONSC 1024

COURT FILE NO: 05-CV-288385PD2

DATE: 20110214

SUPERIOR COURT OF JUSTICE - ONTARIO

RE: Zeljko Rajic, Plaintiff

and

Aaon Atkins, Defendant

BEFORE: Madam Justice Darla A. Wilson

COUNSEL: *Mark Klaiman*, for the Plaintiff

Robert S. Franklin and Lisa Marie Buccella, for the Defendant

HEARD: February 8, 2011

ENDORSEMENT

[1] This action proceeded to trial commencing January 10, 2011, with the jury retiring to consider their verdict on February 8, 2011. Before the jury rendered their verdict, counsel for the defendant brought a motion to have the action dismissed on the basis that the Plaintiff had failed to demonstrate that he had sustained a permanent serious impairment of an important physical, mental or psychological function pursuant to the legislation that was in effect at the time of the accident of June 4, 2003—"Bill 59". The motion that I am asked to decide is commonly known as a "threshold motion".

[2] The relevant portions of the legislation are section 267.5(5) and (15):

267.5(5) Protection from liability, non-pecuniary loss — Despite any other Act and subject to subsection(6), the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for non-pecuniary loss...from bodily injury or death arising directly or indirectly from the use or operation of the automobile, unless as a result of the use or operation of the automobile the injured person has died or has sustained, (a) permanent serious disfigurement; or (b) **permanent serious impairment of an important physical, mental or psychological function.**
[emphasis mine]

267.5(15) Determination at trial; non-pecuniary loss — if no motion is made under subsection (12), the trial judge shall determine for the purpose of subsection (5) whether, as a result of the use or operation of the automobile, the injured person has died or has sustained, (a) permanent serious disfigurement; or (b) permanent serious impairment of an important physical, mental or psychological function.

[3] A consideration by the Court of what constituted permanent and serious impairment commenced with the case of *Meyer v. Bright* (1993), 15 O.R.(3d) 129 (Ont. C.A.), which was decided under the first insurance regime that imposed a threshold, known as the OMPP. Although the threshold was different in that case, the Court of Appeal analyzed what was intended by the legislation and directed trial judges to consider in each case whether the plaintiff has satisfied the onus of establishing on a balance of probabilities that he or she comes within one of the statutory exemptions.

[4] The Court of Appeal set out the steps to be followed in undertaking the analysis.

“We conclude therefore that the appropriate approach in these cases is to answer sequentially the following questions: 1. Has the injured person sustained permanent impairment of a bodily function caused by continuing injury which is physical in nature? 2. If the answer to question number 1 is yes, is the bodily function, which is permanently impaired, an **important** one? 3. If the answer to question number 2 is yes, is the impairment of the important bodily function **serious**?”

The onus of proof to demonstrate on a balance of probabilities that the plaintiff falls within one of the statutory exceptions falls on the plaintiff. Each case, obviously, must be determined according to its particular set of facts and the evidence before the court.

Evidence at the Trial

[5] The Plaintiff, Zeljko Rajic, is 42 years of age and was born in Bosnia. He immigrated to Canada in 1999, where he found employment at Linemar, a company that produces car parts. He worked on the line as a machine operator. He met Danijela and married in 2000 and their son, Alexa, was born in 2001. The plaintiff and his family resided in an apartment in Kitchener at the time of the accident.

[6] He was involved in a prior motor vehicle accident in February of 2000 and according to Mr. Rajic, he was a bit sore, missed one day of work and then had no further problems. In January of 2002, he picked up a 17 pound piece of material at work and sustained an injury to his low back. He filed a WSIB claim and was examined by Dr. Richards, a physician for the company. He was off work for 20 days, and then he gradually returned to his 8 hour shifts, working light duties. That meant that he did not have to do heavy lifting or bending—the pieces that he had to put in to the machine were smaller and lighter. He undertook physiotherapy following the WSIB injury and in fact, he was returning from a physiotherapy treatment when the accident occurred in June of 2003.

[7] The evidence concerning the Plaintiff's physical condition at the time of the car accident and the nature and extent of the back injury from the WSIB incident was contradictory and I will make reference to this issue later in these reasons.

[8] Following the motor vehicle accident of June 4, 2003, the Plaintiff attended at the Guelph General Hospital where he was examined and released. He attended upon his family doctor, Dr. Segedi, a few days later. She prescribed physiotherapy and pain medication and said she would see him again in 10 days. For reasons which were not entirely clear, Mr. Rajic switched family doctors to Dr. Vujnovic, who practises in Toronto.

[9] Since the time of the accident, Mr. Rajic has continued to complain of constant pain in various parts of his body, most notably his neck, left knee, low back, numbness running down both legs and into his feet and headaches. According to Mr. Rajic at trial, none of his injuries have improved since the accident and in fact, some are getting worse. Presently, he cannot sit for longer than 10 minutes and he cannot walk normally. He has to walk very slowly and take little steps. He must use a cane to support himself and he cannot bend. In addition, the Plaintiff testified that he has nightmares, flashbacks from the accident, depression, anxiety, problems concentrating and memory difficulties as a result of the accident.

[10] The Plaintiff testified at trial he has been unable to return to any form of employment, and his marriage broke down and his wife left him due to his problems stemming from the accident. He is unable to clean his apartment and must rely on friends to assist him. He is fearful of driving and must get his friends to drive him to Toronto for his medical appointments. He has been unable to do the usual activities with his son and has not been able to return to playing soccer as he did before the accident. He sees his psychiatrist, Dr. Hanick once per month and he continues to see Dr. Vujnovic twice per month. He takes a great deal of prescription medication, which is of very little assistance. Because of his constant pain, he is able to do very little. Mostly, he stays in his apartment and he does not like to socialize any longer. He testified that the accident has destroyed his life.

Medical Evidence

[11] Dr. Sharma, a specialist in physical and rehabilitation medicine, testified on behalf of the Plaintiff. He examined the Plaintiff in 2008 and at that time, there were no neurological findings or any evidence of muscle spasm. On a physical basis, Dr. Sharma felt the plaintiff could return to his job, but he felt there were psychological factors that were preventing this. He candidly acknowledged that he accepted the history provided by Mr. Rajic as true and accurate and he did not have the benefit of pre-accident clinical notes and records. He was not aware of the status of the WSIB injury at the time of the car accident or whether the Plaintiff continued to work light duties at Linemar.

[12] Dr. Karen Abrams is a psychiatrist retained by the solicitor for the Plaintiff. She assessed Mr. Rajic initially in July 2008 and was provided with a variety of medical reports as well as certain surveillance tapes. She took a history from the Plaintiff and he told her that he was knocked unconscious in the accident. His pain complaints were 15 out of 10 on a scale of 1 to 10. She diagnosed post-traumatic stress disorder [PTSD], a major depressive disorder and a pain

disorder. She did not feel he would return to work and testified that the surveillance tapes did not impact on her diagnosis. She accepted what Mr. Rajic said at face value and did not conduct any validity testing. In her view, his exaggeration can be indicative of a cry for help, as can misstating other things such as his ability to drive.

[13] The other psychiatrist who assessed the Plaintiff was Dr. Wayne Furlong, who was retained by the lawyer for the defendant. He saw Mr. Rajic in July 2010 and read approximately 5000 pages of medical documentation. Dr. Furlong found Mr. Rajic hostile, angry and difficult to interview. He claimed that his pain had not improved and was always 10 out of 10 on the pain scale. Dr. Furlong found a number of the claims made by Mr. Rajic to be odd: for example, he said he developed depression instantly after the motor vehicle accident, which is very atypical; he said he developed a fear of driving and that Dr. Hanick told him not to drive, which is unusual and contrary to psychiatric practice; he said he had nightmares which were all the same, like a replay of a video, which is very odd; he said he had memory problems such that he forgot his own name, which is very unusual for a patient with depression; he could not remember the first letter of the alphabet, which is unbelievable; and he said he often thought of suicide, yet his treating psychiatrist Dr. Hanick did not do a suicide assessment.

[14] Like Dr. Abrams, Dr. Furlong used the DSM IV for diagnosis. However, he did not agree with her diagnosis. He did not believe there was any psychiatric diagnosis possible due to the amount of distortion that was present. Dr. Furlong did not find the Plaintiff to be a reliable historian and he found significant exaggeration and some invention in what he told him. Dr. Furlong expressed the opinion that no doctor can rely on Mr. Rajic's self-report of his complaints or history without some corroboration and he noted that Dr. Abrams did no validity testing. Even if Dr. Abrams were correct in her diagnosis of PTSD, it is usually highly treatable

[15] Dr. Vujnovic became the Plaintiff's family doctor only after the motor vehicle accident. He first assessed Mr. Rajic on June 9, 2003. The plaintiff told him that he had a constant headache, neck pain, back pain and left knee pain as a result of the car accident. Dr. Vujnovic diagnosed a concussion, gave him medications and prescribed physiotherapy. He saw the plaintiff a few more times over the summer and Mr. Rajic complained that he was no better, but worse. Dr. Vujnovic sent him to some specialists: Dr. Hanick a psychiatrist; Dr. Roscoe an orthopaedic surgeon; and Dr. Dimitrakoudis, a neurologist. He never referred him to a pain management programme.

[16] Dr. Vujnovic testified that during the appointments, Mr. Rajic said he was feeling worse. He said he was in constant pain and was depressed. He believed what the plaintiff told him and never questioned whether he was being truthful.

[17] Dr. Vujnovic was not aware of what type of treatment Dr. Hanick was providing to the plaintiff, and he frankly admitted that he ignored any reports that came from doctors or any medical practitioners retained by insurance companies because in his words, "they are all crooks". He never asked the plaintiff about his marital difficulties even though Mr. Rajic told him that he and his wife had separated and Danijela was his patient as well.

[18] Dr Henry Berry has a number of specialties but was qualified as a neurologist in this court. He was retained by the defence and assessed the plaintiff through an interpreter on November 24, 2008. Mr. Rajic told Dr Berry he had constant low back pain, right leg pain, neck pain and headaches, along with occasional left leg pain, as well as anxiety and depression. Dr Berry was aware of the WSIB injury and he noted the plaintiff had prior low back pain which travelled into his right leg.

[19] Dr Berry examined the plaintiff and found no evidence of nerve root compression or muscle atrophy in his legs. He noted spasm of the lower back muscles on the right side, which is a physical sign of pain and is an objective finding. He reviewed the various imaging studies, noting the degenerative changes that were present on the MRI are not serious. There were many inconsistencies and much exaggeration in what the Plaintiff told Dr. Berry. He concluded that the plaintiff's presentation was factitious, which meant that there was no factual basis for the many complaints that he made. He was engaged, according to Dr Berry, in illness behaviour—that is, he had symptoms that were not supported by physical or objective findings. His opinion was that the plaintiff needed to understand that he was capable of a more active life and he needed to get off the multiple medications that he was taking.

[20] Dr. Stephen Gallay was qualified as an orthopaedic surgeon and he was retained by the solicitor for the Plaintiff to provide an opinion on the nature and extent of Mr. Rajic's injuries. He saw the plaintiff on 3 occasions: 2003; 2004 and 2006. He was aware the plaintiff had a WSIB injury 1.5 years prior to the motor vehicle accident in which he suffered a low back injury with right leg discomfort. Following the car accident, the plaintiff described a 50 percent increase in his low back pain as well as the development of left leg pain. He complained of headaches, neck pain, left knee pain and an inability to drive. Dr. Gallay did not note an obvious limp. In 2003, Dr. Gallay diagnosed an exacerbation of his mechanical low back pain.

[21] Dr. Gallay next saw the plaintiff in 2004, at which time Mr. Rajic said he had not returned to work due to his inability to sit for longer than 10 minutes. His diagnosis was the same as in 2003.

[22] He last saw the plaintiff in February 2006. He noted Mr. Rajic had a limp and he complained of pain going into both legs, the right worse than the left. The Plaintiff did not say that his pain levels changed; rather they were constant and disabling. Dr. Gallay noted he had a flat back which is something often seen in patients with back pain. Straight leg raising was limited to 45 degrees, which is an indication of pain. Dr. Gallay did not find any neurological signs of sciatica or radiculopathy. He diagnosed chronic neck and myofascial back pain. He felt the car accident had worsened the plaintiff's back symptoms.

[23] He reviewed the MRI of the lumbar spine that was done in February 2004. It showed a small disc bulge at L3-4 and L4-5. He felt the bulge at L4-5 was likely the source of the back pain because there was a tear of the annulus. This can occur with any sort of force—perhaps a sneeze, a cough or the trauma from an accident—he was not sure when it occurred but said there was a reasonable probability that it occurred during the car accident.

[24] Dr. Gallay made it clear that the orthopaedic findings were as he put it “soft”. There was nothing in the imaging or various scans that explained the plaintiff’s symptoms. He believed there was something else going on, and that the plaintiff might be developing chronic pain, something which he is not qualified to treat. He agreed that the findings of the neurologists, Dr Dimitrakoudis and Dr. Little were inconsistent with his findings. He has not had the opportunity to review any medical documentation since his assessment in 2006.

[25] Dr. Ezra Silverstein was the orthopaedic surgeon who examined Mr. Rajic in February of 2010 at the request of the lawyer for the defendant. He reviewed the medical brief as well as the various imaging studies that have been done. He took a history from the plaintiff and it was notable for the fact that he denied that he had ever experienced pain in his legs or that his prior low back pain arose from an injury.

[26] Dr. Silverstein examined the plaintiff and found that straight leg raising was normal. He noted Mr. Rajic complained of pain in the low back which ran down both legs and went into all his toes with numbness. This is a most unusual finding, according to Dr Silverstein because in cases of a herniated disc, the numbness usually just goes into one toe. Herniated discs cause pain down a leg which follows a certain path depending on which nerve is involved. In addition, patients with a disc herniation develop wasting of the muscle which the nerve supplies and Mr. Rajic had no evidence of muscle bulk loss.

[27] The MRI of February 2004 showed degenerative changes, desiccation and a broad based disc bulge, which according to Dr. Silverstein was of no clinical significance. The plaintiff’s neurological examination was normal, which is what the other doctors found as well. There was no evidence of radiculopathy. In Dr. Silverstein’s opinion, the plaintiff certainly did not have a disc herniation; if he had sustained a herniation in the accident, there would have been immediate symptoms and it would be easily identified on an MRI.

[28] Dr. Silverstein found some of the plaintiff’s complaints were bizarre—for example on a test that did not involve movement of the back, he reported severe back pain. Similarly, he reported pain in the low back while log rolling—this involves the rotation of the leg at the hip and there is no back involvement at all, yet the plaintiff said the manoeuvre caused him excruciating pain. In his opinion, the motor vehicle accident resulted in soft tissue injuries which would not result in any impairment—the accident was merely an exacerbation of his pre-existing low back pain.

The Surveillance Tapes

[29] During the course of the trial, surveillance videos were introduced by the solicitor for the defendant. These tapes were dated April 2005, January 2006, February 2006, April 2006, May 2006, September 2006, May 2007 and July 2007 and were all marked as exhibits at the trial.

[30] These tapes show the Plaintiff walking with his son without a limp, driving to the grocery store with his wife and doing the groceries, carrying what appears to be a 10 pound bag of potatoes, clearing the snow off his car, taking his son to and from school, working under the

hood of his car for approximately 45 minutes, sweeping out his garage and various shopping trips.

[31] The tape that merits specific reference is the one from May 22, 2006. In this tape, the plaintiff is seen driving a truck to his home with a lawn mower in the back of the vehicle. He is observed pulling strongly on the cord of the lawn mower many times to start it. He is seen mowing the lawn, raking the grass, bending down clipping longer grass with long handled clippers, and bending right down to pull weeds. He is seen operating the lawn mower cutting the entire front yard. He is observed raking the lawn at the front and taking the yard waste and putting it in a large green garbage can.

[32] In a nutshell, the surveillance tapes disclose the plaintiff participating in activities he denied at trial he was capable of doing, and which he denied he was able to do to the various doctors who assessed him. When asked about this during his cross-examination, Mr. Rajic was hostile and argumentative. The explanation offered was that he must have been having one of his good days when he was filmed by the investigator. Given the wide-ranging complaints of his pain and resulting disability, this is hardly a satisfactory explanation.

Analysis

[33] All of the experts agree there are no objective findings that would account for the type of complaints Mr. Rajic voices. Dr. Abrams, Dr. Sharma and Dr. Gallay all concur that the Plaintiff's presentation must be due to psychological factors. Indeed, the defence expert Dr. Berry testified that chronic pain by definition is pain that lasts for longer than 6 months with no physical basis for it and that part of chronic pain is the psychological overlay that accompanies it.

[34] Because there is nothing in the medical evidence to explain the severe disability and pain that is described by Mr. Rajic, his credibility is paramount in this action. In discharging his onus to prove that he has sustained a permanent serious impairment of an important physical, mental or psychological function, he must be believed that he is incapable of returning to work and participating in his former activities as a result of injuries from the June 2003 accident. If he fails to satisfy the court on a balance of probabilities, his claim must be dismissed.

[35] At trial, Mr. Rajic was not an impressive witness. While under cross-examination, he was hostile and argumentative and his credibility was seriously undermined. For example, he testified that he has been unable to work normally since the accident; he cannot walk up stairs or bend. He cannot sit for longer than perhaps 10 minutes and cannot stand for any longer than 20-30 minutes. His sworn evidence was that he developed a fear of driving immediately following the accident and did not drive for at least 2 years because of his intense pain and anxiety.

[36] The surveillance tapes show Mr. Rajic on numerous occasions doing things that he denied he was capable of doing: driving a car; going to daycare and picking up his young son; grocery shopping; going to the bank; bending under the hood of his car, apparently doing some sort of maintenance; bending over at the waist for continued periods of time pulling weeds out of the lawn and clipping long grass; bending over at the waist and pulling on the cord of the lawn

mower on multiple occasions to get it started; walking normally, without a limp, without the assistance of a cane; and mowing the lawn for a period of approximately 45 minutes.

[37] On only one occasion during the surveillance periods in May 2007 did Mr. Rajic ever demonstrate a limp. During all of the other occasions that he was observed, he was walking in a normal fluid fashion, without facial grimacing or obvious pain behaviour and he was engaged in what I will describe as the normal day to day activities of someone with a young son. In short, the surveillance tapes cast serious doubt upon the veracity of the evidence of Mr. Rajic concerning the restrictions he has arising from the accident.

[38] The contents of the surveillance tapes stand in stark contrast to the histories provided by Mr. Rajic to the various medical practitioners and indeed to the court during this trial. He portrayed himself as disabled from even the most basic tasks such as feeding himself and washing his hair. I note that the future care costs expert, Ms. Jodi Harendorff, included in her calculations someone to help him dress and undress, wash his hair, make his meals and assist him with eating. As another example, she included the costs of having him transported to his medical appointments as well as the use of a taxi for local transportation and the cost of a driver desensitization programme, all because of his professed fear and inability to drive. Since Ms. Harendorff never met with the Plaintiff nor did she attend at his apartment, she relied on what Mr. Rajic had told a chiropractor in 2006 about what he was prevented from doing as a result of his accident related injuries. The surveillance makes it clear that the plaintiff was already doing many of the activities that he denied he was capable of performing to various medical specialists.

[39] Further, there were many inconsistencies pointed out by the various doctors in the information provided to them by Mr. Rajic. He minimized the significance of the WSIB back injury, describing it as “mild” or “occasional”. He did not tell the physicians he was continuing to receive therapy on a weekly basis at the time of the car accident. To some, he stated that he had pain running into his right leg following the WSIB accident; to others he denied any leg pain before the car accident. The records of Dr. Segedi describe the low back pain from the WSIB injury as “acute” with sciatica running down the right leg. Mr. Rajic told all of the doctors he was unconscious as a result of the collision but the hospital records indicate no loss of consciousness. He reported having a herniated disc in the motor vehicle accident to Dr. Berry but there is no evidence of this in any of the imaging. He never attempted to return to Linemar and try to do a light duty job; nor has he, on the evidence, ever shown any inclination towards finding a different job.

[40] Of greater concern in my view are the inconsistencies noted on testing done by the doctors. Despite his complaints to Dr. Berry of his inability to walk and constant low back pain radiating down his legs, the doctor found no evidence of muscle wasting, which is what one would expect, given those symptoms over a period of 7 years. Dr. Silverstein noted that the plaintiff complained of an increase in his back pain on tests that did not even involve the back. He found no abnormality on straight leg raising and diagnosed soft tissue injuries, an exacerbation of pre-existing back pain from the WSIB incident and certainly not anything that would result in the sorts of impairments the Plaintiff was alleging.

[41] Dr. Furlong found inconsistencies on his testing for psychological injuries. For example, the plaintiff stated that he had severe memory problems as a result of the accident that were getting worse. He professed he could not remember the first letter of the alphabet, nor his own name. Dr. Furlong found this very odd and testified that this sort of difficulty would only be expected in a patient perhaps in the most advanced stages of Alzheimer's. On a memory test for 15 items, Mr. Rajic said he could only recall 6 but Dr. Furlong testified that severely brain damaged people can usually remember 9 items. It was Dr. Furlong's opinion that Mr. Rajic was consciously exaggerating his problems for financial gain. On several occasions, according to Dr. Furlong, Mr. Rajic inquired as to who would be compensating him for his problems.

[42] I did not find the evidence of Dr. Abrams to be of much assistance because she admitted that she simply accepted what the Plaintiff told her without corroboration from other sources, such as other medical records. She testified that she made her diagnosis on the facts as she found them and seemed unconcerned with the inconsistencies in what she was told. She did not, for example, compare what the Plaintiff was shown to be doing on the surveillance tapes with what he told doctors at the time about his level of activity. To Dr. Abrams, this was not important.

[43] Dr. Sharma was candid when he acknowledged that he was not provided with the records of doctors who treated Mr. Rajic before the accident and so he had to rely on what the Plaintiff provided by way of history. He was unaware of the Plaintiff's medical condition at the time of his involvement in the car accident. There were no neurological abnormalities on Dr. Sharma's examination, nor any muscle wasting or muscle spasm. However, Dr. Sharma testified that Mr. Rajic presented and behaved as an invalid.

[44] I found the orthopaedic surgeon, Dr. Gallay, called by the Plaintiff to be an impressive witness. He was knowledgeable in his responses to questions and saw the plaintiff on 3 occasions, the most recent assessment being in 2006. He felt that the accident resulted in an exacerbation of the mechanical low back pain that stemmed from the WSIB injury and that the plaintiff's complaints went beyond orthopaedic based pain. He admitted there were inconsistencies between his findings and those of other doctors.

[45] The opinion of the rehabilitation specialist Jodi Harendorff was of no value at all, given that she never met with the Plaintiff and simply prepared her report based on what Mr. Rajic told a chiropractor in 2006. She had no idea of the Plaintiff's current level of functioning, treatment or needs.

Analysis

[46] Mr. Rajic is an unreliable historian. When impeached on cross-examination, he blamed others such as interpreters, doctors and other health care providers. Comments about exaggeration, psychogenic pain and illness behaviour are contained in numerous medical reports.

[47] I concur with the comment expressed by Dr. Furlong, that the Plaintiff's self-report cannot be used as a basis for a diagnosis because of the many inconsistencies and statements that are clearly not accurate. The opinions of Dr. Sharma and, more importantly, Dr. Abrams rely to a great extent on the truthfulness of what Mr. Rajic reported to them and consequently, I do not

attach significant weight to their opinions. I accept the evidence of Dr. Gallay in terms of the nature of the injury to the Plaintiff's back that is attributable to the car accident. However, he conceded that there is no orthopaedic basis for the types of pain complaints and disability made by Mr. Rajic and if there is a psychological component to his problems that is outside the range of Dr. Gallay's expertise. Casting the Plaintiff's case in its best light, the motor vehicle accident caused an exacerbation of the mechanical low back pain that was initiated by the WSIB incident in January 2002.

[48] When a person makes numerous complaints that are not worthy of belief, how does one determine what is legitimate? I am quite certain that Mr. Rajic has some pain complaints that relate to injuries from the June 2003 accident. His sworn evidence at trial concerning his pain and limitations are inconsistent with the level of function demonstrated on the surveillance tapes. I am not persuaded that they are as disabling as he alleges or that they have required him to cease participation in activities that he formerly enjoyed, including his employment.

[49] In considering the first part of the 3 prong test, I am not persuaded on a balance of probabilities that Mr. Rajic has sustained a permanent serious impairment of an important physical, mental or psychological function. He has failed to discharge the burden of proof upon him and accordingly, the defendant's motion is granted.

D.A. Wilson, J

Release Date: February 14, 2011